

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Rabies Vaccine, Pre-Exposure Prophylaxis (Rabipur or Verorab)

Standalone PGD, separated from the combined travel document

This PGD was previously part of a single combined document covering Japanese encephalitis, rabies and meningococcal ACWY vaccination. That was raised by Pharmacy Plus Health on 21 August 2026: a pharmacy adopting one of the three was signing for all three, including vaccines it may not stock and in which its staff may not be trained. The three have been separated so that each can be adopted, trained for and signed off on its own merits.

This PGD covers PRE-EXPOSURE vaccination only. Post-exposure treatment is not within its scope under any circumstances and is managed through urgent medical assessment, because it may require rabies immunoglobulin and a risk assessment that a community pharmacy cannot perform.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products supplied, as well as BNF advice and any applicable national guidance • All users must be competent in intramuscular vaccination technique and in the recognition and management of anaphylaxis • All users must be competent in travel risk assessment, including access to post-exposure treatment at the destination • All users must be able to explain clearly what pre-exposure vaccination does and does not do, and what the patient must do after any potential exposure • All users must previously have used PGD's to supply or administer medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medicinal products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Clinical condition or situation to which this PGD applies

PGD Indication	Pre-exposure prophylaxis against rabies in individuals at increased risk through travel or occupation. Rabies is an almost invariably fatal viral encephalitis once symptoms develop. Pre-exposure vaccination does not prevent the need for post-exposure treatment, but it simplifies that treatment considerably and removes the need for rabies immunoglobulin,
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	which is expensive and often unavailable in the countries where risk is highest.
Inclusion criteria	- Any age. Both Rabipur and Verorab are licensed for use in all age groups. - Travel to a rabies enzootic area, particularly where any of the following apply: post-exposure treatment and rabies biologics are lacking or in short supply at the destination; the trip involves higher risk activities such as cycling or running; or the stay is longer than one month. - Occupational risk abroad: animal control and wildlife workers, veterinary staff and zoologists working in enzootic areas. - UK-based occupational risk: laboratory staff working with rabies virus, workers at DEFRA-authorized quarantine premises or animal carriers, those who regularly handle bats including voluntarily, and veterinary and technical staff with enhanced occupational risk. - Sufficient time before travel to complete a primary course. - Valid informed consent obtained. - No exclusion criteria present.
Exclusion criteria	- Any actual or possible exposure that has already occurred, including any bite, scratch or lick on broken skin from a mammal in a rabies risk area, however trivial and however long ago. This is a post-exposure situation. Refer for urgent medical assessment the same day and do not attempt to manage it under this PGD. - Confirmed anaphylactic reaction to a previous dose of rabies vaccine or to any component of the product to be used. - For Rabipur specifically: severe egg allergy, because it contains chick embryo cell residues including ovalbumin. Verorab may be a suitable alternative. - For Verorab specifically: hypersensitivity to polymyxin B, streptomycin or neomycin, or to related antibiotics. - Acute severe febrile illness. Postpone until recovered. - Intradermal administration. This PGD covers the intramuscular route only. The intradermal route requires specific technique, is off-label for Rabipur, and is outside the scope of this PGD.
Cautions	- Pregnancy. Give pre-exposure vaccine where the risk of exposure is high and rapid access to post-exposure treatment would be limited, and record the risk assessment. There is no identified harm signal but human data are limited. - Breastfeeding. The same principle applies. No risk to the infant has been identified. - Immunosuppression, including HIV. A full response may not be mounted. Use the conventional three dose schedule rather than the accelerated one, and refer for post-course serology to confirm a protective titre, taken as 0.5 IU/mL or above. - Bleeding disorders, thrombocytopenia or anticoagulation. Give by deep subcutaneous injection rather than intramuscularly. - Chloroquine antimalarial prophylaxis suppresses the response to intradermally administered rabies vaccine. This PGD uses the intramuscular route only, which is not affected, but the interaction is recorded here because patients may ask. - Where rabies vaccine is co-administered with Japanese encephalitis vaccine on the accelerated JE schedule, a faster decline in rabies antibody has been observed. Bear this in mind

	when advising on booster timing for a traveller who has had both.
Anaphylaxis and observation	- Adrenaline 1:1000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance. - All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life support training. - The patient should remain on the premises for observation after vaccination in line with the pharmacy SOP. - Procedures must be in place to prevent injury from a vasovagal faint, which is common around vaccination, particularly in adolescents.
Actions if patient is excluded or declines treatment	- Discuss the reason for exclusion with the patient and ensure they understand it. - Advise on alternative options and how these can be accessed, for example the GP, a travel clinic, or a specialist service. - Document the reason for exclusion, the advice given and the decision reached. - Inform or refer to the GP as appropriate, and where the exclusion is time critical make the urgency of that referral explicit to the patient.

Details of the medicine

Name, form and strength of medicine	- Rabipur (Bavarian Nordic), purified chick embryo cell vaccine. Powder and solvent, reconstituted to 1.0 mL per dose. Thiomersal free. Contains chick embryo cell residues and traces of neomycin, chlortetracycline and amphotericin B. - Verorab (Sanofi), purified Vero cell rabies vaccine. Powder and solvent, reconstituted to 0.5 mL per dose. - Note that the dose volumes differ between the two products, 1.0 mL for Rabipur and 0.5 mL for Verorab. Confirm which product is held before drawing up. Record the product, batch number and expiry for every dose.
Legal category	POM
Storage	Store at 2°C to 8°C in the original packaging, protected from light. Do not freeze. Use immediately after reconstitution.
Route/method of administration	- Intramuscular injection only under this PGD. Deltoid in those aged 2 years and over; anterolateral thigh in younger children. - Never give into the gluteal muscle: the immune response is unreliable from that site. - In bleeding disorders, thrombocytopenia or anticoagulation, give by deep subcutaneous injection. - May be given at the same visit as other travel vaccines at a separate site, preferably a different limb, or at least 2.5 cm apart in the same limb.
Dose and frequency of administration	- Rabipur 1.0 mL per dose. Verorab 0.5 mL per dose. - Conventional primary course, preferred where time allows: three doses on day 0, day 7 and day 28. The third dose may be brought forward to day 21 where there is insufficient time before travel. - Accelerated primary course, only where there is insufficient time for the conventional course: three doses on day 0, day 3 and day 7, followed by a further dose at one year if travel to high risk

	areas continues. - Boosters are not routinely recommended for most travellers. A single booster may be considered, after risk assessment, where the primary course was completed more than a year ago and the individual is travelling again to an enzootic area. - Those with frequent unrecognised exposure risk, such as bat handlers, should have a reinforcing dose at one year and then boosters every 3 to 5 years or as guided by serology. - Laboratory staff working routinely with live virus should have antibody titres checked every 6 months to determine booster timing. This is outside pharmacy scope; refer to occupational health. - A two dose pre-exposure regimen on day 0 and day 7 now appears in both product SPCs. UKHSA and JCVI guidance on its use is awaited, so this PGD deliberately follows the Green Book three dose schedules. Do not use the two dose regimen under this PGD until the position is confirmed and this document is reissued.
Quantity to be administered and/or supplied	One dose per patient per attendance, in accordance with the schedule above.
Adverse effects	Very common: headache, dizziness, rash, injection site reactions, malaise, fatigue, asthenia and fever. Common: lymphadenopathy, reduced appetite, gastrointestinal upset, urticaria, myalgia and arthralgia. Uncommon or rare: hypersensitivity, paraesthesia, chills, dyspnoea. Very rare or reported post-marketing: anaphylaxis, encephalitis, Guillain-Barre syndrome, syncope, vertigo, angioedema, and for Verorab sudden hearing loss which may persist. Causality is not established for the neurological events. Consult the current SPC for the product used.
Yellow card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP with whom the individual is registered - name of healthcare practitioner - name and brand of vaccine, batch number and expiry date - date of administration, dose, route and anatomical site used - where a course is involved, the dose number and the date the next dose is due - advice given, including advice given if excluded or if the patient declines - details of any adverse reaction and the action taken - that the vaccine was administered via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed.

What the patient must be told about exposure. This is the most important part of the consultation.

- Pre-exposure vaccination does not remove the need for urgent medical attention after any potential exposure. Say this plainly and check the patient has understood it. A patient who believes they are now protected and does not seek help after a bite is worse off than one who was never vaccinated at all.

- Any bite, scratch or lick on broken skin or a mucous membrane from any mammal, including dogs, cats, monkeys and bats, in a rabies risk area, is a potential exposure. Bats in the UK also carry rabies-related viruses.
- Immediate first aid matters and should be done at once, before seeking help: wash the wound thoroughly with soap and running water for several minutes, apply a disinfectant such as 40 to 70% alcohol or povidone iodine, and cover with a simple dressing.
- Seek medical assessment locally and urgently. Do not wait until returning to the UK.
- Because they have had pre-exposure vaccination, they will need only two further doses of vaccine, on day 0 and day 3, and will almost certainly not need rabies immunoglobulin. Without pre-exposure vaccination they would need four doses and, for a higher risk exposure, immunoglobulin as well, which is often not available where it is most needed.
- Carry a record of the vaccine brand and the dates of each dose while travelling. It changes what treatment they are given abroad.
- The incubation period is usually 3 to 12 weeks but has ranged from a few days to many years, so advice should be sought even for an exposure some time ago.
- Vaccination is not a substitute for avoiding animal contact. Advise against approaching, feeding or handling animals, including apparently friendly domestic animals and injured wildlife.

Patient information

- Give the patient the manufacturer's patient information leaflet.
- Explain the schedule and the date each subsequent dose is due, and give this in writing.
- Go through the exposure advice above and record that it was given. Consider providing it as a printed card the patient can keep with their travel documents.
- Counsel on the common local and systemic reactions.

References

- UKHSA. Immunisation against infectious disease, the Green Book, chapter 27, Rabies. Revised 19 May 2023.
- Summary of Product Characteristics for Rabipur and Verorab, current versions, electronic Medicines Compendium.
- NaTHNaC TravelHealthPro. Rabies disease page and country information pages.
- UKHSA. Rabies post-exposure treatment guidelines, current version.
- Resuscitation Council UK. Emergency treatment of anaphylaxis, current guidance.

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
21/08/2026	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		21/08/2026
Chris Pilkington	Pharmacist GPhC: 2046322		21/08/2026

Change history

Version number	Change details	Date
001	New standalone PGD. Separated from the combined Japanese encephalitis, rabies and meningococcal ACWY document following the query from Pharmacy Plus Health on 21 August 2026. Content rebuilt from Green Book chapter 27 and the current Rabipur and Verorab SPCs. Notable points the combined document did not carry: the different dose volumes of the two products, 1.0 mL for Rabipur against 0.5 mL for Verorab; egg allergy as a Rabipur-specific exclusion with Verorab as the alternative; the exclusion of any already-occurred exposure as a post-exposure situation requiring urgent referral; and a full patient counselling section on what to do after a bite. The newly licensed two dose pre-exposure regimen is deliberately excluded pending UKHSA and JCVI guidance.	21st August 2026